

Questions for the Oncology Team

Princess Margaret Cancer Centre • Colorectal surveillance & ATM-variant care • Prepared for the next appointment

Three questions, each tied to a decision we actually need to make. The answers will shape both her treatment choices and how far from Toronto we can plan to be.

1 | What does "continual chemo" for the ATM target actually mean?

You mentioned a treatment that targets the ATM defect but said it would require continual chemotherapy. We need to understand exactly what that is:

- Is it **cytotoxic chemotherapy of the same type that hospitalized me** after barely one cycle — or a **targeted agent** such as a PARP inhibitor or ATR inhibitor?
- If it is a targeted agent: is it **oral**, and how does its **side-effect profile compare** to the chemo I could not tolerate?
- My decision between pursuing treatment and choosing hospice / MAID depends entirely on whether this would repeat that experience. I don't want to rule it out — or count on it — based on the word "chemo" if the reality is different.

2 | Was the severe reaction explained, and does it change what is safe for me?

The chemotherapy that put me in hospital after one cycle was, I believe, a fluoropyrimidine (capecitabine).

- Have I been tested for a **DPYD gene variant** (DPD deficiency)? If not, should I be?
- Would that result explain the severity of what happened, and does it change which drugs are safe for me going forward?
- Does it interact in any way with the **ATM finding**?

3 | Keeping 4×/year surveillance — and whether it can ever travel with me

I want to **keep CT surveillance four times a year**. I am not comfortable stepping down to twice a year; I want the earliest possible chance of catching a recurrence. Given that:

- Is a **contrast CT required at every one** of the four intervals, or can some intervals be covered by a **CEA blood test** (which I could have drawn anywhere)?
- We plan to base in **Mexico City** for parts of the year. Would you **accept a CT done at Médica Sur (JCI-accredited, CDMX) to your protocol, with the full DICOM images sent to you to read** — or do

you require every scan be done at PMH? Specifically, could the **November and February** scans be done there so we can avoid Toronto winter?

- I'm willing to have **one scan done at both Médica Sur and PMH** within a short window, so you can confirm the images are comparable before relying on the outside centre.
- If acceptable, what **exact CT protocol** (contrast, phases, anatomical coverage, slice thickness) should Médica Sur follow so it is directly comparable to my PMH scans?

A discussion aid prepared with the patient, not medical advice. The oncology team's guidance governs all clinical decisions.